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NORTHFIELD HEALTH

Emergency Department Potassium Repletion Standing Protocol

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Section 4.1 Background and indications

Hypokalemia in emergency presentations can cause cardiac rhythm disturbances and muscle weakness. This protocol lets trained nursing staff start potassium replacement without waiting for an individual physician order.

Section 4.2.1 Nurse standing authority

In emergent hypokalemia, a registered nurse at Tier 2 or higher may dispense and administer potassium chloride intravenously under this standing order, without prior physician co-signature, for adult patients with a confirmed serum potassium below 3.2 mEq/L. Rates must follow CARD-GUIDE-2024: no more than 10 mEq/hr by peripheral line and no more than 20 mEq/hr by central line with a pump and telemetry. Physician co-signature must be obtained within 4 hours of starting, in line with NFH-REG-2024 Section 14-8.

Section 4.2.2 Renal clearance validation

Standing nurse administration under Section 4.2.1 is contingent on verified renal clearance. A serum creatinine below 1.8 mg/dL, obtained within the preceding 4 hours, must be documented in the electronic medical record. If renal clearance is absent, or if acute anuria or oliguria is observed, bedside consultation with an emergency attending physician is mandatory before the infusion is primed.

Section 4.3 Exclusions

Patients under 18 years of age are excluded from standing-order administration. All pediatric electrolyte orders require an attending physician order and pharmacy double-verification. Patients with known renal failure are also excluded.

If the patient is transferred out of the emergency department to a general floor, direct attending physician countersignature is required before the infusion bag is hung. See HOSP-BYLAW-88 Section 11.8.

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